

Convalescent Snakes & Ladders: Facilitation Guide

Purpose of the session

This artefact is designed to support structured examination of the epistemic demands of post-acute recovery. It is not a simulation, assessment, or empathy exercise. Its educational value lies in the interpretive pressure it creates and the facilitated discussion that follows. The goal of facilitation is **not** to resolve uncertainty during play, but to hold it long enough for participants to examine how recovery reasoning unfolds under constraint.

Recommended setting

- **Group size:** small groups (typically 4–12 participants)
- **Format:** facilitated teaching session
- **Engagement:** participants interact with the artefact individually - on their own devices - while sharing the same session context (Note the game is not optimised for use on mobile phones, this is part of the deliberate constraints).
- **Assessment:** not suitable for summative or evaluative use

Facilitator stance

During play, facilitators should:

- avoid explaining future rules or outcomes
- avoid reassuring participants that uncertainty will “make sense later”
- avoid framing frustration as error or failure

Intervention during play should be limited to clarifying immediate procedural steps required to continue.

The facilitator’s role is to **hold uncertainty**, not to resolve it.

Timing (indicative)

- **Orientation (5–10 minutes):** minimal framing; explain only what is required to begin
- **Engagement (10 - 15 minutes):** participants play independently, in parallel
- **Discussion (30–45 minutes):** structured examination of interpretive experience

Premature explanation undermines the artefact’s pedagogical function.

Lines of inquiry for discussion

Facilitated discussion should attend to **specific interpretive moments** rather than general impressions. Useful lines of inquiry include:

- Where did bodily signals, effort, or outcomes become difficult to interpret over time?
- When did it become unclear what counted as normal, concerning, or actionable?
- Which events only made sense retrospectively, after consequences emerged?
- At what points did responsibility increase without a corresponding increase in clarity, support, or control?
- What kinds of reasoning were required to continue despite limited guidance or delayed feedback?
- How does this compare with what patients are typically oriented to expect or manage at discharge?

Discussion should focus on **reasoning processes under constraint**, not on emotional response or personal disclosure.

Common facilitation pitfalls

The following reduce the effectiveness of the artefact:

- resolving uncertainty too early
- treating frustration as a design flaw
- moralising the experience (blame, empathy, professionalism)
- rushing to solutions or “what should be done”

The artefact is intended to **surface a gap**, not to close it.

What the artefact is (and is not)

- **It is:** a provocation that renders epistemic labour visible and discussable
- **It is not:** a model of best practice, a behavioural intervention, or a clinical guideline

Learning emerges through examining how assumptions formed in acute care fail to map onto post-acute recovery conditions.

Supplementary materials

Detailed thematic mapping and technical documentation are available as supplementary materials for educators who wish to explore the artefact further.